6.18 Chronic Pancreatitis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The pain never fully goes away." • Drinks alcohol daily **Objective** • Epigastric pain for a long time • Pancreatic calcification on imaging	**ACTUAL** Chronic pain related to pancreatic damage as evidenced by persistent abdominal pain for months.	• Pain at a level that allows daily function.	• Agree a regular analgesic plan with the physician. • Support him to stop drinking and refer to services. • Teach non-drug methods of pain relief. • Watch for analgesic dependence.	• Goal reached when pain is manageable.
Subjective • "My stool is greasy and floats." **Objective** • Steatorrhea • Lost 5 kg	**ACTUAL** Inadequate nutritional intake related to malabsorption as evidenced by steatorrhea and weight loss.	• Weight stays steady.	• Give pancreatic enzymes with every meal and snack, as prescribed. • Advise small, frequent meals. • Refer to the dietitian. • Weigh weekly.	• Met if weight is stable.
Subjective *No specific complaint* **Objective** • Islet cells lost • Fasting glucose rising	**RISK** Risk for ineffective blood glucose pattern self-management as evidenced by risk factors: pancreatic endocrine insufficiency and limited knowledge of glucose monitoring.	• Blood glucose stays on target.	• Check blood glucose. • Teach the symptoms of high and low blood glucose. • Refer to the diabetes educator if diabetes develops.	• Achieved if glucose remains controlled.

* **References for this care plan:** [32], [35], [57], [81]. Full entries are listed in the References section of the CPMN book.